

CognaSync

Behavioral pattern recognition for outpatient mental health care

Executive Summary

Prepared for John Graham (Sunbelt Holdings) and Jonathan Cohen (Idealab Arizona) · May 2026

WHAT COGNASYNC IS

CognaSync is a behavioral pattern-tracking platform that pairs a patient-facing daily check-in with a provider-facing clinical summary, so the clinician walks into the appointment already knowing what has actually been happening in the patient's life between visits. The product surfaces patterns in self-reported data — mood, sleep, stress, medication adherence, substance use, and behavioral signals — without ever crossing into diagnosis or medication recommendation.

CLINICAL SAFETY ARCHITECTURE

Every AI output is governed by four non-negotiable rules baked into the system prompt and enforced programmatically: never diagnose, never advise medication changes, describe data rather than clinical meaning, and route to crisis resources rather than engage with self-harm content.

All quantitative scores — stability, stim load, sleep disruption, crash risk, nervous system load — are computed deterministically in code before any AI call, so the model never invents or recomputes numbers; it only references values passed to it as structured data.

Crisis language is intercepted before it ever reaches the model, and every generated output is post-processed against a forbidden-language list that catches diagnostic phrasing, medication recommendations, and certainty overclaims that the model might otherwise let slip.

WHERE WE ARE TODAY

Platform built and operational: dual-interface web product with patient daily check-in (basic and advanced modes), provider dashboard with patient summaries and threshold alerts, four AI output modes, deterministic scoring engine, symptom-correlation detection across eleven tracked variables, substance-use pattern detection, and a provider-only interpersonal safety signal detector. Built on Flask, React, Supabase, and the Anthropic API. *[Customize: current pilot status, design partners under discussion, any provider conversations underway.]*

THE NEXT TWELVE MONTHS

Months 1–3. Sign first clinical design partner — an Arizona behavioral health practice or outpatient psychiatric group — and run a structured pilot to validate provider workflow integration and clinical utility.

Months 4–6. Refine the provider interface against real clinical feedback. Sign a second design partner. Formalize regulatory and security posture (HIPAA BAA infrastructure, SaMD positioning).

Months 7–12. Convert design partners to paying providers. Validate willingness to pay at the practice level. Begin structured conversations with Arizona health systems (Valleywise, Banner Health) about institutional pilots.

SPECIFIC ASK FROM THIS CONVERSATION

One introduction from John Graham to behavioral health leadership at Valleywise Health Foundation, for an exploratory pilot conversation. **One structural conversation** with Jonathan Cohen and the Idealab Arizona team about whether and how the studio's resources — network, GTM, ASU connectivity — can engage with a built, non-ASU-affiliated company in the medical/AI vertical. Capital is welcome but secondary to network and clinical relationships.